

Question:

A 5-year-old boy is brought to the physician by his mother because he does not “listen to her” anymore. The mother also reports that her son cannot concentrate on any tasks lasting longer than just a few minutes. Teachers at his preschool report that the patient is more active compared to other preschoolers, frequently interrupts or bothers other children, and is very forgetful. Last year the patient was expelled from another preschool for hitting his teacher and his classmates when he did not get what he wanted and for being disruptive during classes. He was born at term via vaginal delivery and has been healthy except for 3 episodes of acute otitis media at the age of 2 years. He has met all developmental milestones. His mother has major depressive disorder and his father has Graves' disease. He appears healthy and well nourished. Examination shows that the patient does not seem to listen when spoken to directly. The remainder of the examination shows no abnormalities. Which of the following is the most appropriate next step in treatment?

Answer Choices:

- A. Behavior therapy
- B. Methimazole
- C. Fluoxetine
- D. Hearing aids

Reasoning + Answer:

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Finding Reasoning Paths :

- 1. **Neurodevelopmental issues**: The patient exhibits hyperactivity , impulsivity , and inattention , which are hallmark symptoms of Attention Deficit Hyperactivity Disorder (ADHD). This could be due to genetic or environmental factors .
- 2. **Maternal mental health**: The mother has major depressive disorder , which could influence the child 's behavior . However , ADHD is a primary diagnosis in this case .
- 3. **Overactivity and impulsivity**: These symptoms are consistent with ADHD rather than hearing issues or thyroid dysfunction .
- 4. **Head circumference and developmental concerns**: The patient has met all developmental milestones , and his head circumference is within normal limits . This rules out structural abnormalities like hydrocephalus .

Reasoning Process :

- 1. **Symptoms Analysis** :
 - The patient presents with hyperactivity , impulsivity , and inattention , which are core symptoms of ADHD . ADHD is characterized by difficulty in sustaining attention , hyperactivity , impulsivity , and distractibility .
 - The significant inattention and impulsivity , combined with the report of the child not being able to concentrate on tasks for more than a few minutes , strongly suggest ADHD rather than developmental delays or sensory issues .
- 2. **Developmental Milestones** :
 - The patient has met all developmental milestones , which reduces the likelihood of intellectual disability or significant developmental disorders .
- 3. **Family History** :
 - The mother 's major depressive disorder could theoretically influence the child 's behavior , but ADHD is still the most likely primary diagnosis given the symptoms . Depression alone does not explain the child 's hyperactivity and impulsivity .
- 4. **Overactivity and Impulsivity** :
 - The child 's overactivity and impulsivity are better explained by ADHD than by hearing issues (e.g. , conductive hearing loss) or thyroid dysfunction (e.g. , hyperthyroidism). Hearing and thyroid issues would typically present with more specific symptoms , such as difficulty with hearing (in conductive hearing loss) or thyroid-related symptoms like fatigue or weight changes .
- 5. **Neurodevelopmental Testing** :
 - While an initial referral for neurodevelopmental testing could be considered , the symptoms strongly suggest ADHD . ADHD can often be diagnosed without specialized testing if the symptoms are clear and persistent .

Conclusion :

The patient is likely to have **Attention Deficit Hyperactivity Disorder (ADHD)** based on the described symptoms of hyperactivity , impulsivity , and inattention . The most appropriate next step in treatment is **behavior therapy** , which can be combined with medication if needed , but behavior therapy is typically recommended as first-line treatment for ADHD .

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<answer>

Behavior therapy

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